



Nexus Vascular Imaging

Vascular Ultrasound Report
22 Bay Road Sandringham
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Patient Name: Alexandre Deev

Date of Birth: 28/08/1954

Report Date: 20/07/2026

Study: Bilateral Lower Limb Venous Ultrasound

Accession: WS-1356

Referring Doctor: Dr Lachlan Maddock

UR Number: UR 100076

Exam Date: 20/07/2026

Sonographer: Sam Johnson AMS 12704

Clinical Indication

Bilateral skin changes and varicose veins.

Findings

RIGHT:

Normal phasic flow in the common femoral vein suggests no ilio caval obstruction.

Incompetent saphenofemoral junction with reflux from the superficial epigastric vein source into a posterior thigh tributary and recurrent great saphenous vein. The posterior thigh tributary is straight for 12 cm and becomes extrafascial distally.

Proximal great saphenous vein is absent.

Reflux extends through the recurrent great saphenous vein to associated thigh and calf tributaries.

Deep vein assessment suboptimal due to swelling. No deep venous incompetence or thrombosis identified.

Incompetent medial calf perforator.

LEFT:

Normal phasic flow in the common femoral vein suggests no ilio caval obstruction.

Incompetent thigh great saphenous vein with reflux to medial thigh and posterior calf tributaries. Great saphenous vein is straight and subfascial in the thigh. Segmental incompetence of the anterior accessory vein. Incompetent anterior thigh circumflex vein, source of reflux via an incompetent perforator.

Occlusive wall thickening in the mid calf great saphenous vein.

Deep vein assessment suboptimal due to swelling. No deep venous incompetence or thrombosis identified.

Incompetent medial thigh and medial calf perforators noted.

Impression

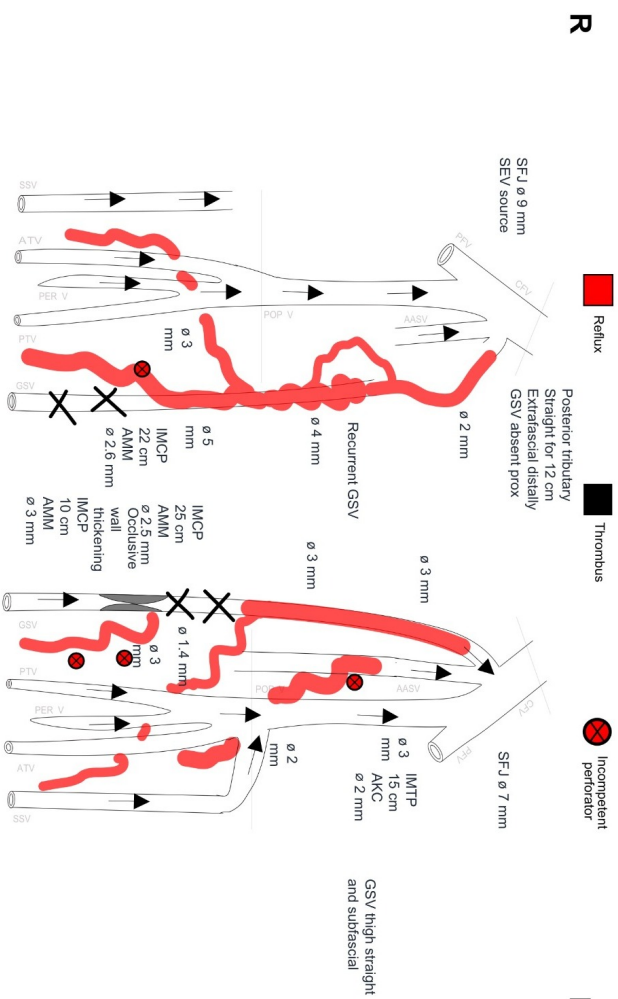
Right: recurrent great saphenous vein incompetence with associated tributary and perforator reflux.

Left: great saphenous vein and tributary reflux. Perforator incompetence.

Mr Sam Farah MBBS(Hons) MMed FRACS
Vascular Surgeon

To: Dr Lachlan Maddock

BILATERAL LOWER LIMB VENOUS ULTRASOUND



COMMENTS:

Suboptimal deep vein assessment